Health Insurance

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Poll (No Right Answer)

The reason that politicians want to intervene in the health insurance market is to correct a market failure, not to redistribute to the needy.

- A. True
- B. False
- C. Uncertain

Poll

Can the government pay for universal healthcare by taxing the top 1%?

- A. Yes
- B. No

Poll (No Right Answer)

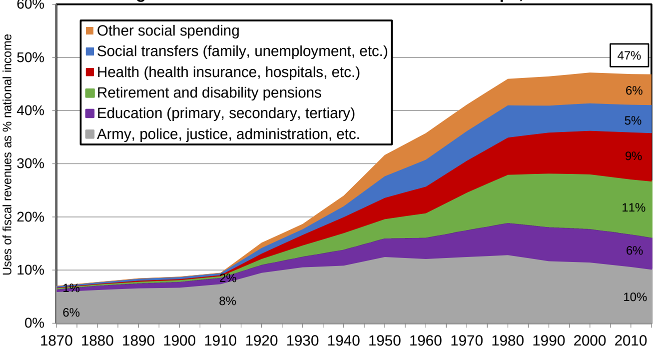
Should the government fund health care for all Americans?

- A. Yes
- B. No

Motivation

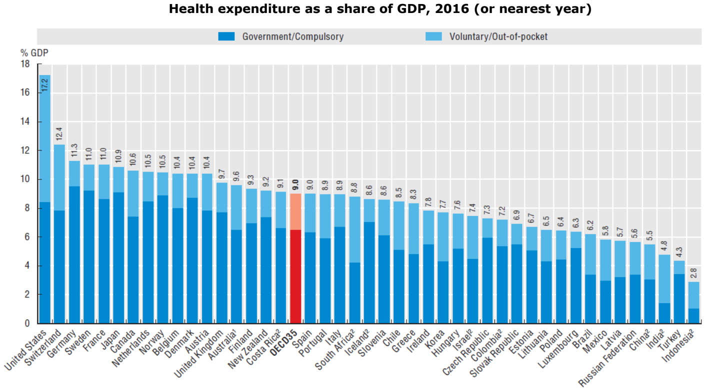
- Health care is costly (modern medicine is hi-tech) and everybody demands it (often considered a right)
- Advanced economies spend about 9% of their GDP on health care [up from 2-3% in 1950]
- Low income families would not be able to afford much health care on their own
- $\Rightarrow$ In all countries, government plays major role in funding health care
- U.S. health care system has significant issues:
    - US health care is expensive (18% of GDP vs 9% on average in other OECD countries)
    - A significant fraction of population (9-10%) is uninsured

Figure 10.15. The rise of the social State in Europe, 1870-2015



Interpretation. In 2015, fiscal revenues represented 47% of national income on average in Western Europe et were used as follows: 10% of national income for regalian expenditure (army, police, justice, general administration, basic infrastructure: roads, etc.); 6% for education; 11% for pensions; 9% for health; 5% for social transfers (other than pensions); 6% for other social spending (housing, etc.). Before 1914, regalian expenditure absorbed almost all fiscal revenues. **Note.** The evolution depicted here is the average of Germany, France, Britain and Sweden (see figure 10.14). Sources and séries: see piketty.pse.ens.fr/ideology.

Health spending was 9% of GDP on average in the OECD, ranging from 4.3% in Turkey to 17.2% in the United States



Note: Expenditure excludes investments, unless otherwise stated.
1. Australian expenditure estimates exclude all expenditure for residential aged care facilities in welfare (social) services.
2. Includes investments.

***Source*: Health at a Glance 2017.**

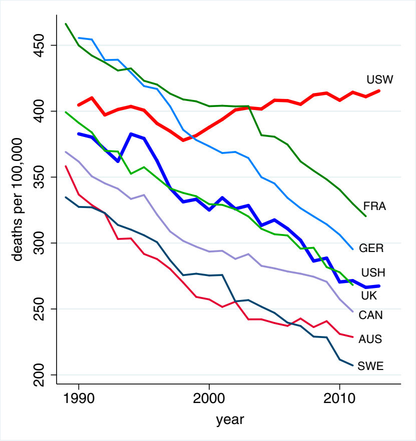


Fig. 1. All-cause mortality, ages 45–54 for US White non-Hispanics (USW), US Hispanics (USH), and six comparison countries: France (FRA), Germany (GER), the United Kingdom (UK), Canada (CAN), Australia (AUS), and Sweden (SWE). Source: Case and Deaton (2015)

Universal Health Insurance

- All OECD countries except the US provide universal health insurance funded by taxation
- Individuals who get sick can have health care paid for by the government
- Government either directly controls doctors/hospitals (like National Health Service in the UK) or government reimburses private health care providers (like in France or Germany)
- Government controls costs and limits health-care over-consumption through:
  1. Regulation (govt picks allowed treatments based on cost effectiveness, bargains for prices, rations care in some cases)
  2. Patient co-payments (patients share part of the cost)

US Health Insurance

- US has a mix of public and private insurance: As of 2024 [web link]
  1. Government provided insurance [36% of population]
     - Medicare for the elderly (65+) = 19% of pop
     - Medicaid for the poor = 18% of pop
     - Other (mostly veterans benefits) = 4% of pop
  2. Privately provided insurance [66% of population]
     - Employer provided health insurance = 54%
     - Individual purchases (mostly Obamacare exchanges) = 11%
  3. Uninsured [8% of pop.] (16% before Obamacare)

Employers Provided Insurance

- Covers half of the US population. Started after WW2 when health care costs were low.
- Employer level insurance allows risk pooling across employees
- But cost has grown enormously: \$27K/family premium in 2025 (\$9.3K single)
- Workers ultimately bear the cost in the form of reduced wages [as employers care about total labor cost = wage + benefits]
- Saez and Zucman (2019): Health insurance functions like a "privatized poll tax" (i.e., a flat-dollar-amount "tax") on workers, since the security guard pays as much as the CEO
- On Obamacare exchanges, individual purchase is subsidized based on family income (see below)

Nongroup Insurance

- Nongroup direct insurance market: The market through which individuals or families buy insurance directly rather than through a group, such as the workplace.
- The nongroup insurance market was not a well-functioning market before Obamacare
- Those in the worst health (pre-existing conditions) were often unable to obtain coverage (or could only obtain it at an incredibly high price)
- Even without pre-existing conditions, there was adverse selection
- Obamacare exchanges has drastically changed the nongroup market by forbidding pricing/discrimination based on preexisting conditions and mandating health insurance (but the fine for non-coverage repealed in 2019+)

Medicare

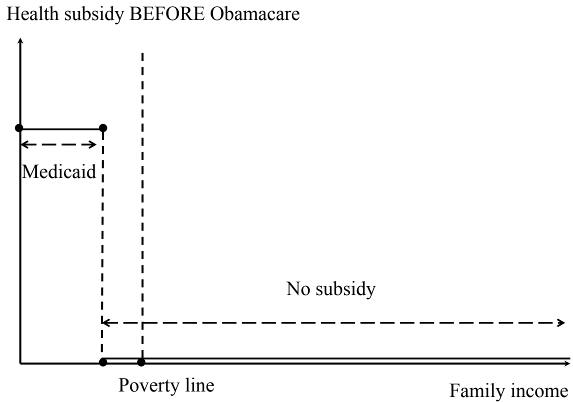
- Started in 1965 as a universal health insurance system for the elderly, and for nonelderly on disability insurance.
- Federal program that provides health insurance to all people over age 65 or disabled
- Every citizen who has worked for 10 years (or their spouse) is eligible
- Financed with an uncapped payroll tax totaling 2.9% (along with general revenue)
- Physician reimbursement fairly generous (but not as high as private insurance)

Medicaid

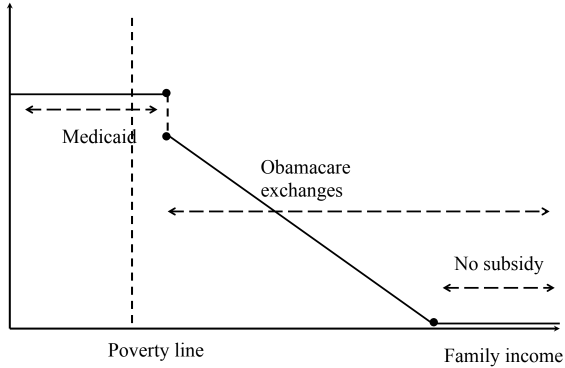
- Provides health care for the poor (means-tested benefit)
- Financed from general revenues by both Fed and State
- Targets welfare recipients, low income kids and elderly (for non-Medicare costs such as long-term care)
- 70% of recipients are mothers/kids but 66% of expenditure goes to long-term care for elderly/disabled.
- Doctor reimbursement low $\Rightarrow$ some docs refuse Medicaid
- Big variation across states in Medicaid generosity (costs are shared between state/feds)
- Program eligibility criteria have been expanded over time (higher incomes allowed): Obamacare substantially expands Medicaid to reduce the fraction uninsured [but not all states do it]

Obamacare (Affordable Care Act, ACA)

- Three-pronged system starts in 2014
  1. "Community rating": Bans pre-existing conditions exclusion, health-based pricing
  2. Mandate: Forces individuals (and large employers with 50+ employees starting in 2015/6) to buy health insurance [else they pay a fine]. Individual fine gone in 2019+
  3. Free/subsidized insurance for low-income families: (a) Medicaid expansion up to 138% of poverty line paid by Feds at 90% and (b) subsidized health insurance purchases in Obamacare exchanges up to 400% of poverty line [high deductibles and copays in exchanged while none on Medicaid]
- Funded primarily with surtax on rich
- Starts trying to control costs (indeed cost increases have slowed down in recent years)


Health subsidy BEFORE Obamacare
Medicaid
No subsidy
Poverty line
Family income

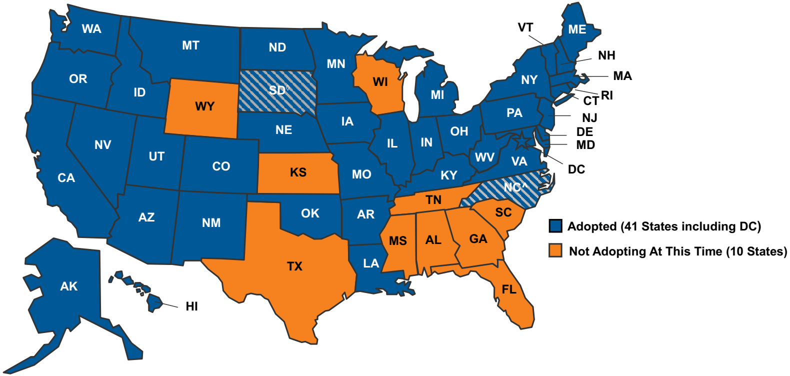
Health subsidy after Obamacare in Medicaid Expansion States



Legal Challenges to Obamacare

- July 2012 court case: Is the mandate constitutional?
- Ruling: yes, but Feds cannot force States to expand Medicaid $\Rightarrow$ Many states (including TX, FL) decided to opt-out of the Medicaid expansion [even though Fed govt pays 90%]
- Consequence: Coverage gap because people below 100% of poverty cannot access subsidized Obamacare exchanges
- States moving slowly to accept Medicaid expansion mostly through referenda, 41 states (including DC) as of 2026, [web link]
  1. Can the Feds set up exchanges if states don't do it themselves? [Ruling: yes, July 2015]
  2. There are still pending court challenges to Obamacare

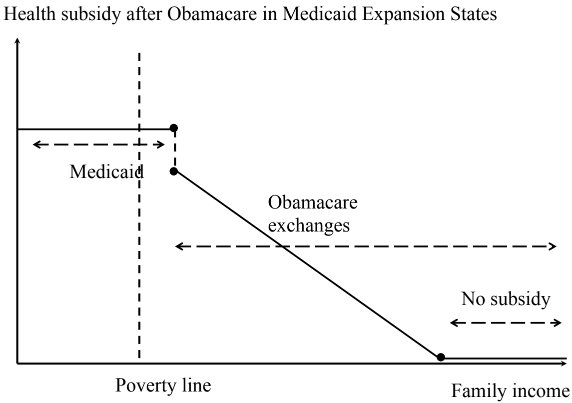
Status of State Medicaid Expansion Decisions

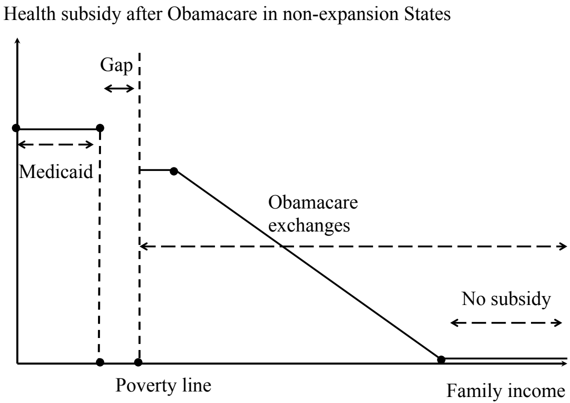


NOTES: Current status for each state is based on KFF tracking and analysis of state activity. ◊Expansion is adopted but not yet implemented in SD. ^Implementation of Medicaid Expansion is contingent on appropriations in the SFY 2023-2024 biennial budget in NC. See link below for additional state-specific notes.

SOURCE: "Status of State Action on the Medicaid Expansion Decision," KFF State Health Facts, updated March 27, 2023. https://www.kff.org/health-reform/state-indicator/state-activity-around-expanding-medicaid-under-the-affordable-care-act/

KFF


Health subsidy after Obamacare in Medicaid Expansion States
Medicaid
Obamacare
exchanges
No subsidy
Poverty line
Family income


Health subsidy after Obamacare in non-expansion States
Gap
Medicaid
Obamacare
exchanges
No subsidy
Poverty line
Family income

The Uninsured

- Fraction of individuals uninsured fell by nearly 50% with Obamacare: from 15% of population in 2013 down to 9% in 2017.
- Three groups of uninsured:
  1. Undocumented immigrants (no access to Medicaid or Obamacare subsidized exchanges) $\simeq$ 10m
  2. Low income people who don't qualify for Medicaid and Obamacare insurance subsidies in states that did not expand Medicaid [possible that more states will expand]
  3. People who did not sign up for Obamacare exchange (used to pay the fine, no fine in 2019+), poor people who qualify for Medicaid but haven't taken up benefits
- Key issue: uninsured face prohibitive health care costs [high prices from hospitals] so don't get care or go bankrupt with health care debt [no market serving uninsured has arisen]

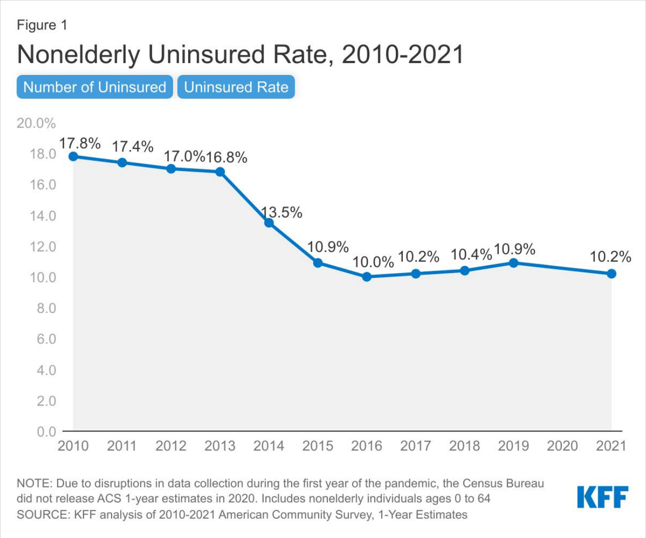
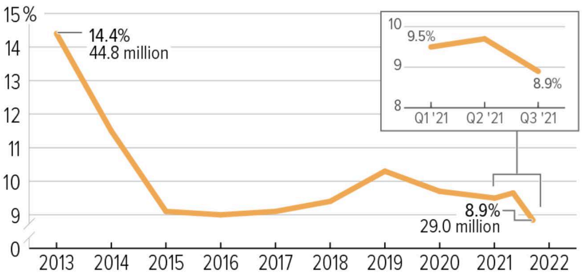


Figure 1
Nonelderly Uninsured Rate, 2010-2021
Number of Uninsured
Uninsured Rate
20.0%
18.0
16.0
14.0
12.0
10.0
8.0
6.0
4.0
2.0
0.0
17.8%
17.4%
17.0%
16.8%
13.5%
10.9%
10.0%
10.2%
10.4%
10.9%
10.2%
2010
2011
2012
2013
2014
2015
2016
2017
2018
2019
2020
2021
NOTE: Due to disruptions in data collection during the first year of the pandemic, the Census Bureau did not release ACS 1-year estimates in 2020. Includes nonelderly individuals ages 0 to 64
SOURCE: KFF analysis of 2010-2021 American Community Survey, 1-Year Estimates
KFF

Uninsured Rate Stabilized During Pandemic and Data Suggest Recent Declines in 2021

Uninsured rate by year, all ages



Note: Estimates of uninsured rates in 2021 reflect quarterly data through Quarter 3 of 2021. All other years are annual data.

Source: National Health Interview Survey's Health Insurance Coverage Reports, 2013-2020; Health Insurance Coverage: Early Release of Quarterly Estimates From the National Health Interview Survey, July 2020–September 2021.

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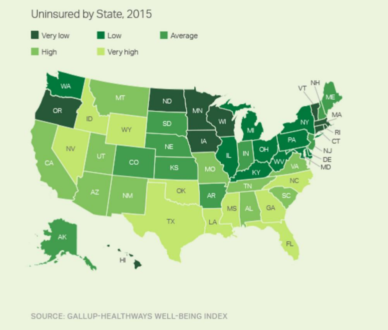
Coverage Gains Vary by State



State	% Uninsured 2013	% Uninsured 2015	Expanded Medicaid
California	21.6	11.8	Yes
Colorado	17.0	10.3	Yes
Florida	22.1	15.7	No
Illinois	15.5	8.7	Yes
Kentucky	20.4	7.5	Yes
Massachusetts	4.9	3.5	Yes
New York	12.6	8.6	Yes
Oregon	19.4	7.3	Yes
Texas	27.0	22.3	No
Virginia	13.3	12.6	No

Is Universal Health Care Desirable?

- Health care is expensive (even in countries which control costs) $\Rightarrow$ Poor cannot afford all health care on their own
- People face different health risks (pre-existing conditions) $\Rightarrow$ Those facing high health risks face high insurance costs in private market
- Should government insure people for health risks? No moral hazard if health risks outside people's control (age, genetics). Moral hazard if health risks due to choices (diet, exercise)
- Virtually all OECD countries answer yes and provide universal health care
  - Not providing universal health care can lead to adverse selection, if private insurers cannot observe risks or cannot charge based on risks $\Rightarrow$ Even those with low risks may not get actuarially fair insurance
  - In all cases (private and public), health insurance needs to deal with moral hazard (over-provision, over-consumption)

Status Quo vs. Public Option vs. Medicare For All

- Right-of-center view appears to be to maintain the status quo, after trying but failing to repeal ACA in 2017.
- Key tension: The "community rating" part of the ACA (i.e., forbidding pricing/discrimination based on preexisting conditions) is very popular. And the economics of adverse selection makes it hard to keep only the community rating part.
- Explanation: Community rating is a force for adverse selection/unraveling (i.e., healthy people don't buy insurance). To prevent unraveling, the ACA has the individual/employer mandate: everyone must be insured. And to prevent financial burden on low earners who must now buy insurance, ACA gives them subsidies or covers them for free via Medicaid.
- Community rating + mandated insurance + subsidies and Medicaid = ACA

Status Quo vs. Public Option vs. Medicare For All

- Two left-of-center views: Public Option or Medicare For All
- Public Option: Brings uninsured to zero by putting version of Medicare on the ACA exchanges, automatically enrolling uninsured in Medicaid or the Public Option, and expand ACA subsidies. Reduces employer-sponsored insurance (ESI) by 10% after eliminating ESI firewall. Most still covered by private insurance. Costs $1.5T over next ten years.
- Eliminating ESI firewall: Employees can go onto exchanges even if employer offers coverage.

Status Quo vs. Public Option vs. Medicare For All

- Medicare For All: Brings uninsured to zero by giving everyone a version of Medicare. Puts private insurance out of business (including Medicare Advantage?). Optional: eliminate co-pays and deductibles and dramatically expand benefits. May cost $15 (if no expansion of benefits) to 35 trillion (if large expansion of benefits) over ten years. Phase-in: 3-10 years.
- Maximum additional revenue that can be raised from top 1%: $10 trillion over ten years (Saez and Zucman 2019)
- Arguments made for Public Option: Choice, competition, queues, political viability, transition path.
- Arguments made for M4A: Cost efficiencies, under-insurance, right, quality of care, progressivity (but only if funded by progressive tax, not by per-employee tax).

Effect of Health Care on Utilization and Health: Oregon Medicaid Health Insurance Experiment

- In 2008, Oregon had a limited Medicaid budget $\Rightarrow$ used lottery to select individuals on waitlist to be given a chance to apply for Medicaid insurance coverage
- 30,000 "lottery winners" (treatment group) out of 90,000 participants (lottery losers are control group)
  - Not all winners received coverage. Some non-winners later received insurance on their own
  - But it is still the case that winning the lottery increases probability of having health insurance by 29 percentage points
- Finkelstein et al. (2012) use lottery as instrument to estimate causal effect of insurance coverage itself
  - Two way to report the results:
  - ITT (intention to treat): just compare winners and losers
  - LATE (local average treatment effect): Inflate estimates by 1/[difference in fraction insured between winners and losers]$=1/.29=3.5$

Oregon Medicaid Health Insurance Experiment

- Data sources: admin data from hospitals, credit reporting data, and survey responses regarding utilization, health, and financial outcomes
- Key results: winning the Medicaid lottery leads to:
  1. higher health care utilization (including primary and preventive care as well as hospitalizations)
  2. lower out-of-pocket medical expenditures and medical debt (including fewer bills sent to collection agencies for unpaid debt)
  3. better self-reported physical and mental health

Table V: Health Care Utilization (Survey Data)

	Extensive Margin (Any)				Total Utilization (Number)			
	Control Mean	ITT	LATE	p-values	Control Mean	ITT	LATE	p-values
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Prescription drugs currently	0.637	0.025	0.088	[0.002]	2.318	0.100	0.347	[0.049]
	(0.481)	(0.0083)	(0.029)	{0.005}	(2.878)	(0.051)	(0.176)	{0.137}
Outpatient visits last six months	0.574	0.062	0.212	[<0.0001]	1.914	0.314	1.083	[<0.0001]
	(0.494)	(0.0074)	(0.025)	{<0.0001}	(3.087)	(0.054)	(0.182)	{<0.0001}
ER visits last six months	0.261	0.0065	0.022	[0.335]	0.47	0.0074	0.026	[0.645]
	(0.439)	(0.0067)	(0.023)	{0.547}	(1.037)	(0.016)	(0.056)	{0.643}
Inpatient Hospital admissions last six months	0.072	0.0022	0.0077	[0.572]	0.097	0.0062	0.021	[0.311]
	(0.259)	(0.0040)	(0.014)	{0.570}	(0.4)	(0.0062)	(0.021)	{0.510}
Standardized treatment effect		0.050	0.173	[<0.0001]		0.040	0.137	[0.0003]
		(0.011)	(0.036)			(0.011)	(0.038)	
Annual spending [a]					3,156	226	778	[0.037]
						(108)	(371)	

Source: Finkelstein et al. 2012

Table VIII: Financial Strain (Survey Data)

	Control Mean	ITT	LATE	p-values
	(1)	(2)	(3)	(4)
Any out of pocket medical expenses, last six months	0.555	-0.058	-0.200	[<0.0001]
	(0.497)	(0.0077)	(0.026)	{<0.0001}
Owe money for medical expenses currently	0.597	-0.052	-0.180	[<0.0001]
	(0.491)	(0.0076)	(0.026)	{<0.0001}
Borrowed money or skipped other bills to pay medical bills, last six	0.364	-0.045	-0.154	[<0.0001]
	(0.481)	(0.0073)	(0.025)	{<0.0001}
Refused treatment bc of medical debt, last six months	0.081	-0.011	-0.036	[0.01]
	(0.273)	(0.0041)	(0.014)	{0.01}
Standardized treatment effect		-0.089	-0.305	[<0.0001]
		(0.010)	(0.035)	

Source: Finkelstein et al. 2012

Table IX: Health

	Control Mean	ITT	LATE	p-values
	(1)	(2)	(3)	(4)
Panel A: Administrative data				
Alive	0.992	0.00032	0.0013	[0.638]
	(0.092)	(0.00068)	(0.0027)	
Panel B: Survey Data				
Self reported health good / very good / excellent (not fair or poor)	0.548	0.039	0.133	[<0.0001]
	(0.498)	(0.0076)	(0.026)	{<0.0001}
Self reported health not poor (fair, good, very good, or excellent)	0.86	0.029	0.099	[<0.0001]
	(0.347)	(0.0051)	(0.018)	{<0.0001}
Health about the same or gotten better over last six months	0.714	0.033	0.113	[<0.0001]
	(0.452)	(0.0067)	(0.023)	{<0.0001}
# of days physical health good, past 30 days*	21.862	0.381	1.317	[0.019]
	(10.384)	(0.162)	(0.563)	{0.018}
# days poor physical or mental health did not impair usual activity, past 30 days*	20.329	0.459	1.585	[0.009]
	(10.939)	(0.175)	(0.606)	{0.015}
# of days mental health good, past 30 days*	18.738	0.603	2.082	[0.001]
	(11.445)	(0.184)	(0.64)	{0.003}
Did not screen positive for depression, last two weeks	0.671	0.023	0.078	[0.001]
	(0.470)	(0.0071)	(0.025)	{0.003}
Standardized treatment effect		0.059	0.203	[<0.0001]
		(0.011)	(0.039)	

Source: Finkelstein et al. 2012

Table X: Potential Mechanisms for Improved Health (Survey Data)

	Control Mean	ITT	LATE	p-values
	(1)	(2)	(3)	(4)
Panel A: Access to care				
Have usual place of clinic-based care	0.499	0.099	0.339	[<0.0001]
	(0.500)	(0.0080)	(0.027)	{<0.0001}
Have personal doctor	0.490	0.081	0.280	[<0.0001]
	(0.500)	(0.0077)	(0.026)	{<0.0001}
Got all needed medical care, last six months	0.684	0.069	0.239	[<0.0001]
	(0.465)	(0.0063)	(0.022)	{<0.0001}
Got all needed drugs, last six months	0.765	0.056	0.195	[<0.0001]
	(0.424)	(0.0055)	(0.019)	{<0.0001}
Didn't use ER for non-emergency, last six months	0.916	-0.0011	-0.0037	[0.804]
	(0.278)	(0.0043)	(0.015)	{0.804}
Standardized treatment effect		0.128	0.440	[<0.0001]
		(0.0084)	(0.029)	

Source: Finkelstein et al. 2012

Effect of Medicare on Health

- Medicare becomes available when you turn 65 $\Rightarrow$ Can do a regression discontinuity design to see what happens when you cross age 65 threshold. Two papers use this strategy:
  1. Card-Dobkin-Maestas "The Impact of Nearly Universal Insurance Coverage on Health Care Utilization and Health: Evidence from Medicare" AER 2008
     - Examines impacts across groups; with an interest in evaluating impacts on inequality in utilization
  2. Card-Dobkin-Maestas "Does Medicare Save Lives?" QJE'09
     - Examines impacts on outcomes (mortality following hospital admission)
- Basic idea is to draw graphs of outcomes based on age for various groups
  - The discontinuity at 65 captures short-term changes in health care utilization and mortality from shift from < 65 to > 65

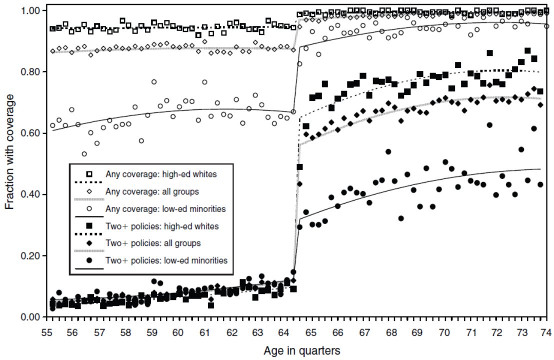


FIGURE 1. COVERAGE BY ANY INSURANCE AND BY TWO OR MORE POLICIES, BY AGE AND DEMOGRAPHIC GROUP

First stage: sharp increase in coverage; more for disadvantaged
(From NHIS; age measured in quarters) FIGURE 1

9

Source: David Card et al (2008)

Hospital discharge data (CA, FL, NY 1992-2002), ages 60-70

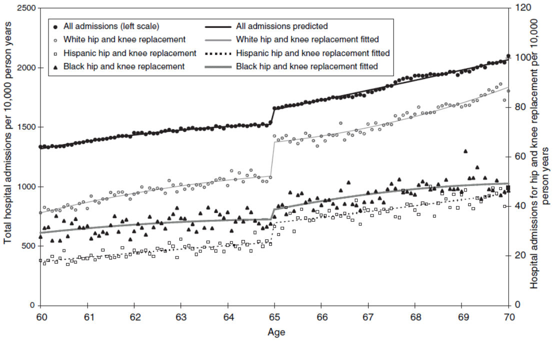


FIGURE 3. HOSPITAL ADMISSION RATES BY RACE/ETHNICITY

Increase is driven by discretionary medical care, diagnostic heart treatments.

13

Source: David Card et al (2008)

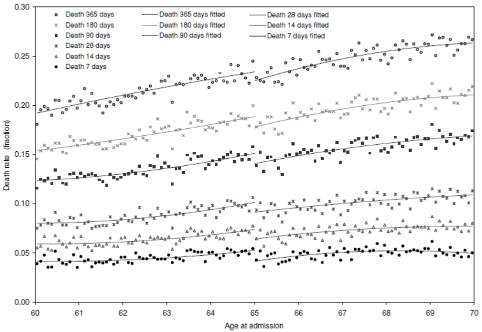


FIGURE VI
Patient Mortality Rates over Different Follow-Up Intervals

Nontrivial decrease in mortality.

22

Source: David Card et al (2008)

Effects of Medicare on Health

- Big increase in health insurance coverage, especially for disadvantaged groups
- Big increase in health care utilization
- Visible decrease in mortality after admission for conditions requiring Emergency Room (ER) immediate hospitalization (so that likelihood of going to hospital is the same before 65 and after 65)
- $\Rightarrow$ Medicare health insurance does save lives

Optimal Health Insurance: Provider Side

- Preceding analysis of optimal insurance assumes patient makes entire healthcare decision
- This assumed a passive doctor, in the sense that doctor provides whatever treatment patient requested
- Reality is closer to the opposite
- Incorporating supply side issues is important in understanding health insurance
- Question: choice of payment schemes for physician
- Retrospective (fee-for-service) vs. prospective (diagnosis based fixed payments)

Optimal Health Insurance: Provider Side

- Intuition: if patient doesn't choose level of care, healthcare may be inefficiently high
- If physician is compensated for all costs $\Rightarrow$ it is in his/her interest to do lots of procedures (e.g. too many C-section births)

Optimal Health Insurance: Provider Side Model

- Payment for physician services is $P = \alpha + \beta \cdot c$
- α=fixed cost payment for a given diagnosis
- β=payment for proportional costs c (tests, nurses)
- Various methods of payment (α, β):
  1. Fee-for-service $(\alpha = 0, \beta > 1)$: No fixed payment for practice, but insurance company pays full cost of all visits to doctor + a surcharge.
  2. Diagnosis based payment $(\alpha > 0, \beta = 0)$: varying by type and # of patients but not services rendered

Optimal Health Insurance: Provider Side

- General trend has been toward higher α, lower β
- Private market has shifted from FFS to HMO (Health Maintenance Organizations) capitation schemes [where insurer pays a fixed amount per patient no matter what the health costs are].
- Example, Kaiser receives a flat amount per person enrolled based on age/gender
- Medicare/Medicaid shifted in 1980s to a prospective payment scheme.
- Tradeoff: lower β provides incentives for doctors to provide less services. But then they may provide too little care.
- $\Rightarrow$ Lower costs, but complaints of lower quality of care

Evidence: Payment Schemes and Physician Behavior

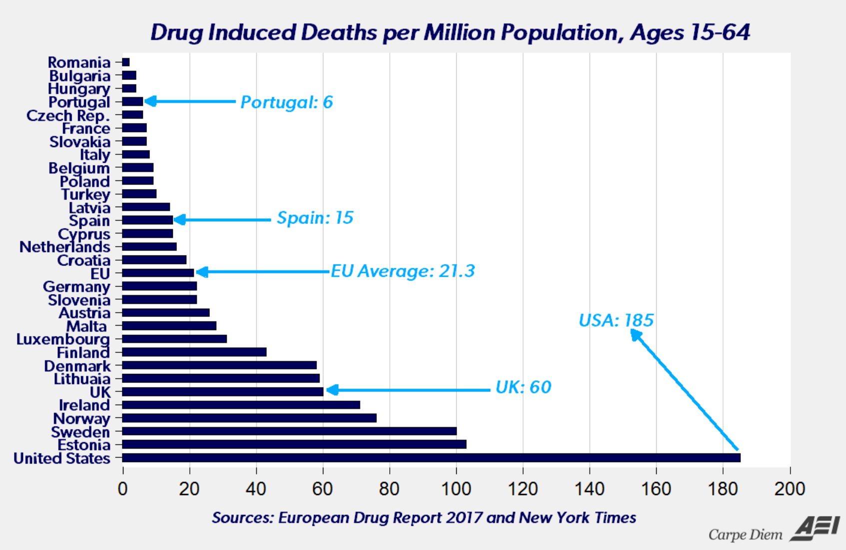
- In 1983, Medicare moved from retrospective reimbursement to prospective reimbursement.
- Prospective payment system (PPS) is Medicare's system for reimbursing hospitals based on nationally standardized payments for specific diagnoses.
- All diagnoses for hospital admissions were grouped into Diagnosis Related Groups (DRGs).
- Government reimbursed a fixed amount per DRG. More severe DRGs received higher reimbursement.

Evidence: Payment Schemes and Physician Behavior

- Cutler (1993) finds that PPS led to:
  1. A reduction in treatment intensity. For example, the average length of hospital stay for elderly patients fell by 1.3 days
  2. No adverse impact on patient outcomes despite the reduction in treatment intensity
- Evidence that doctors put some weight on profits
- Suggests they are practicing "flat of the curve" medicine: too much treatment before.
- Cost growth slowed dramatically in the five years after PPS but then accelerated again.

Opioid Epidemic and Unintended Consequences

- Late 1990s, big pharma pushed opioid pain killers aggressively
- Encouraged doctors to prescribe them (patients love them in the short-run but often get addicted)
- $\Rightarrow$ Led to misuse and addicted then turned to heroin and fantanyl (80% of current addicts started with prescription opioids). US now has 1.5m opioid addicts.
- 70K people/year die from overdoses (5% death rate/year for addicts). 10 times more deaths than in EU relative to pop
- $\Rightarrow$ US is shifting from "addiction is a crime" to "addiction is a health care problem"
  - $\Rightarrow$ Overdose death rates vary tremendously from 6/million in Portugal, 60/million in UK or Sweden, to 185/million in the US [web link]
  - $\Rightarrow$ Portugal decriminalized drugs and deployed health care solution $\Rightarrow$ drop in overdose deaths (more modest decrease in addiction rate)

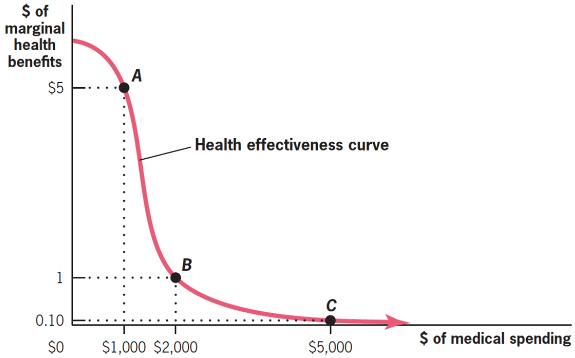

Drug Induced Deaths per Million Population, Ages 15-64
Romania
Bulgaria
Hungary
Portugal
Czech Rep.
France
Slovakia
Italy
Belgium
Poland
Turkey
Latvia
Spain
Cyprus
Netherlands
Croatia
EU
Germany
Slovenia
Austria
Malta
Luxembourg
Finland
Denmark
Lithuaia
UK
Ireland
Norway
Sweden
Estonia
United States
Portugal: 6
Spain: 15
EU Average: 21.3
USA: 185
UK: 60
0
20
40
60
80
100
120
140
160
180
200
Sources: European Drug Report 2017 and New York Times
Carpe Diem

Technology Growth and Health Care Growth

- Health care technology contributes to rising life expectancy
- Many new technologies have modest health effects and are very costly and yet are adopted because Medicare/Private insurance accept any health effective treatment
  - $\Rightarrow$ fuels the development of new technologies, especially testing which leads to growing costs and over-treatment
- Countries which are the most successful at containing costs choose to use only the cost effective new treatments: reduces costs while having very little effect on health outcomes
- From social marginal utility perspective, US health care system spends too much on the insured (where marginal value of care is small) and spends too little on the uninsured (where marginal value of care is high)
- Suggest key US health policy challenges are to (a) cover more of the uninsured, and (b) reduce non-cost-effective health spending (can raise wages in the process)

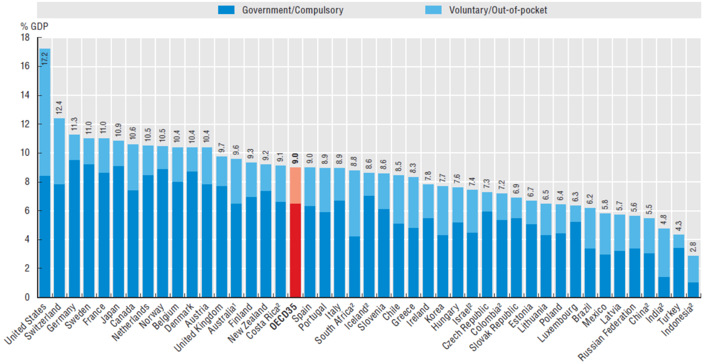
15.2

The "Flat of the Curve"



Health spending was 9% of GDP on average in the OECD, ranging from 4.3% in Turkey to 17.2% in the United States

Health expenditure as a share of GDP, 2016 (or nearest year)



Note: Expenditure excludes investments, unless otherwise stated.
1. Australian expenditure estimates exclude all expenditure for residential aged care facilities in welfare (social) services.
2. Includes investments.

***Source*: Health at a Glance 2017.**

Poll (No Right Answer)

The reason that politicians want to intervene in the health insurance market is to correct a market failure, not to redistribute to the needy.

- A. True
- B. False
- C. Uncertain

Poll

Can the government pay for universal healthcare by taxing the top 1%?

- A. Yes
- B. No

Poll (No Right Answer)

Should the government fund health care for all Americans?

- A. Yes
- B. No

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